

WHITMORE, Harold James

DOB: March 8, 1945 | Age: 75 yrs | Sex: Male | MRN: SMC-04471932

Encounter Date: April 19, 2020 | Provider: Thomas Reed, MD — Pulmonology | Doc ID: SMC-2020-0419-ADM

Hospital Admission & Course

COVID-19 Pneumonia with Hypoxemic Respiratory Failure**HPI**

75-year-old high-risk man (HFrEF, COPD, T2DM, CKD, CAD) with 6 days of fever, dry cough, fatigue, and progressive dyspnea. SARS-CoV-2 PCR positive. Presents with hypoxemia.

Vital Signs

BP (mmHg)	HR (bpm)	RR	Temp	SpO2	Weight	BMI
132/80	96 irreg	28	38.6 C	86% RA → 93% on 4 L	93.0 kg	30.3

Admission Labs

Test	Result	Units	Reference Range	Flag
SARS-CoV-2 PCR	Detected		Not detected	H
C-reactive protein	112	mg/L	< 5	H
D-dimer	1.9	mcg/mL FEU	< 0.5	H
Ferritin	880	ng/mL	30 - 400	H
Lymphocytes (absolute)	0.7	x10 ⁹ /L	1.0 - 3.0	L
Creatinine	1.7	mg/dL	0.7 - 1.3	H
eGFR (CKD-EPI)	41	mL/min/1.73 m ²	> 60	L
Troponin I	0.05	ng/mL	< 0.04	H

Chest CT (04/19/2020)

Bilateral peripheral ground-glass opacities with areas of consolidation, lower-lobe predominant — typical pattern for COVID-19 pneumonia. Background emphysema. No pulmonary embolism on CT angiography. Small bilateral effusions.

Hospital Course (Days 1-9)

Required up to 6 L/min oxygen via nasal cannula; no invasive ventilation. Treated with dexamethasone 6 mg daily x10 days and a 5-day course of remdesivir. Prophylactic-to-therapeutic anticoagulation managed around home apixaban (held briefly, resumed). Hyperglycemia from steroids managed with sliding-scale + basal insulin; metformin and empagliflozin held during acute illness (sick-day rules; SGLT2i held re ketoacidosis risk). Gradually weaned to room air by day 8. Renal function returned to baseline (eGFR 45).

Discharge Plan

- Discharged day 9, SpO2 94% on room air at rest, 90% on ambulation — home oxygen PRN with weaning.
- Resume metformin and empagliflozin once eating/well and renal function stable (resumed at discharge).
- Resume full home cardiac, anticoagulant, and respiratory regimen.
- Pulmonary follow-up in 2-4 weeks; repeat chest imaging in 8-12 weeks. Convalescent monitoring for post-COVID dyspnea.